

HOW TO OPEN A PRIVATE HOME CARE BUSINESS IN GEORGIA

A Georgia-licensed Private Home Care Provider — delivering non-medical personal care (hands-on help with daily living), companionship and sitter services, and home management, so seniors and adults with disabilities can live safely and independently at home.

A step-by-step startup and operations guide

Licensing (DCH/HFRD via GAHLES) · Screening & training · Getting paid · Operations

Regulatory basis: The Rules and Regulations for Private Home Care Providers, Ga. Comp. R. & Regs. Chapter 111-8-65, under O.C.G.A. § 31-7-300 et seq.; licensed and overseen by the Georgia Department of Community Health (DCH), Healthcare Facility Regulation Division (HFRD). This is a NON-MEDICAL license — personal care, companion/sitter, and home management, but not skilled nursing. No Certificate of Need is required. Funding: private pay; Georgia Medicaid home and community-based waivers (CCSP and SOURCE) where applicable; Veterans' benefits; and long-term-care insurance.

How to Use This Guide

This guide walks you from 'I want to start a home care business' to 'I am licensed, serving clients, and getting paid' in Georgia. The big picture is simple: in Georgia, non-medical home care is delivered by a Private Home Care Provider (PHCP) licensed by the Department of Community Health. Unlike some states that split companionship from hands-on care, a Georgia PHCP license lets you provide personal care — real hands-on help with bathing, dressing, and other activities of daily living — alongside companionship, sitter services, and home management. You do not need a Certificate of Need, and you do not need to be a nurse; you need a clean, complete license application, screened and trained caregivers, and a registered nurse to handle care planning and training where the rules require it. Skilled nursing (injections, wound care, medication administration) stays outside your scope — that is home health, a different and much heavier license.

Read Parts 1 through 4 before you spend real money — they tell you what you are allowed to do and exactly how to get licensed. Parts 5 and 6 are about revenue: where clients come from and how you get paid, including Georgia's Medicaid waivers and its twice-a-year application window. Parts 7 through 9 keep you compliant once you are open. The two worked examples show how it comes together in a metro market, and the contacts, glossary, and references at the end are your quick-reference desk. Verify every fee, figure, and phone number before you rely on it — requirements change.

The one-paragraph version

Form your business and get an EIN and insurance. Line up a registered nurse. Apply for a Private Home Care Provider license through Georgia's GAHLES portal; fingerprint every owner of 10%+, your administrator, and your caregivers through GCHEXS. Get your provisional license, start serving clients, and pass your on-site survey to earn your regular license. Bill private-pay clients from day one; to bill Medicaid, enroll in GAMMIS, credential for the CCSP or SOURCE waiver (applications open only in March and September, and your license must be 9 months old first), and clock every visit through Netsmart EVV. Stay in scope, screen and train everyone, supervise on schedule, and document everything.

The Path at a Glance

Here is the whole journey as a numbered path. Later parts expand each step.

1. **Decide your scope:** personal care + companion/sitter + home management, non-medical. No skilled nursing.
2. **Form your entity** with the Georgia Secretary of State, get an EIN, register any trade name at your county Superior Court, and get general and professional liability insurance.
3. **Engage a Georgia-licensed registered nurse** to complete initial service plans for medically frail clients and to direct caregiver training.
4. **Write your policies and procedures** to Chapter 111-8-65 (this package is built for that).
5. **Register in GCHEXS** and fingerprint all 10%+ owners, the administrator, and every direct-care employee.
6. **Apply for your PHCP license through GAHLES** (gahles.dch.ga.gov), upload your documents, and pay the fee.
7. **Receive your provisional license**, begin serving clients, and prepare for your on-site survey.
8. **Pass the on-site survey** before the one-year provisional period ends to earn your regular license.
9. **Start with private-pay clients;** to bill Medicaid, enroll in GAMMIS, credential for CCSP/SOURCE (March or September), and set up Netsmart EVV.
10. **Operate compliantly:** supervise on the 92/122-day schedule, evaluate staff annually, renew your license on time, and keep documenting.

Part 1 — What This Business Is

The license you are getting

In Georgia, a Private Home Care Provider (PHCP) is any entity that provides private home care services in a client's residence. 'Private home care services' means direct care in the home and, under Chapter 111-8-65, can include three things: nursing services, personal care tasks, and companion or sitter tasks. Your non-medical business provides the second and third — personal care and companion/sitter — plus home management, and leaves nursing services out. You are licensed and surveyed by the Department of Community Health's Healthcare Facility Regulation Division (HFRD). A Certificate of Need is not required for a PHCP, which is a major reason so many people start here rather than with a home health agency.

What you can do

- Personal care tasks: hands-on help with activities of daily living — bathing, dressing, grooming, toileting, transferring, ambulation, and eating — delivered by trained Personal Care Assistants.
- Companion and sitter tasks: companionship and supervision, transportation and escort, meal preparation and serving, and household tasks.
- Home management: light housekeeping, laundry, shopping, and similar homemaker activities.

Where the line is

You do not provide skilled nursing — no injections, wound care, or medication administration. Your staff may remind clients to take self-administered medications, but they never administer them. If a client needs skilled care, you document it, tell the client, and refer to a home health agency or physician. Note one important nuance: when you serve a client who is medically frail or compromised, a Georgia-licensed registered nurse must complete the initial service plan and provide supervision. That nurse is doing care planning and oversight — not delivering skilled nursing — so your business stays non-medical while meeting the rule.

Part 2 — Getting Licensed with DCH / HFRD

Georgia moved its licensing online. As of 2025, Private Home Care Provider applications are filed through GAHLES — the Georgia Healthcare Licensing, Enforcement, and Surveys portal at gahles.dch.ga.gov — which replaced the old GAMMIS application path. Communications come from hfrd.applicationswaivers@dch.ga.gov, so watch that inbox.

How the license works

1. **Submit your application on GAHLES** with a description of your services, your service area, your policies and procedures, proof of insurance, and your background-check documentation. Pay the license fee (verify the current amount on the application — third-party guides commonly cite about \$350, but confirm it on the GAHLES fee schedule before you budget).
2. **Get your provisional license.** This is the license you start with. You may begin serving clients as soon as you receive it. It is good for one year and cannot be renewed.
3. **Become operational.** You must actually be serving clients during the provisional year so the state can survey a working agency.
4. **Pass your on-site survey.** Before the provisional license expires, HFRD conducts an on-site survey — reviewing your policies in action, client records, staff, and home visits. Pass it and you receive your regular license.
5. **Renew annually.** About two months before expiration the state sends a renewal notice and fee schedule. Pay on time — miss the deadline and you face a certified notice, a higher 'involuntary' rate, and, if unresolved within 30 days, closure.

Accreditation can lighten your surveys

Under Rule 111-8-65-.08, the Department may exempt an accredited provider from periodic inspection. CHAP is approved to conduct initial licensure surveys for Georgia private home care providers. Accreditation is optional and adds cost, but it can streamline your survey burden as you grow.

Part 3 — Background Screening & Training

Background checks through GCHEXS

Georgia runs fingerprint-based background checks through GCHEXS — the Georgia Criminal History Electronic System — covering both a state (GCIC) and national (FBI) check. You register your agency as a facility in GCHEXS (email VoluntaryBackgroundCheck@dch.ga.gov for access), then screen everyone the rule requires: every owner holding 10% or more, your administrator, and every direct-care employee, before they have contact with clients. No one who has been shown to have abused, neglected, exploited, or seriously harmed a person may serve. You will also complete a Records Check Application and a Personal Identification Affidavit.

Training your caregivers

Georgia is specific about training, and this is where new agencies most often stumble. Personal Care Assistants (the staff who do hands-on care) must qualify one of four ways: a nurse-aide training and competency program (42 CFR Part 483, Subpart D); a nurse-aide competency exam the Department recognizes; an approved credentialing program; or your own 40-hour training program run under the direction of a registered nurse. If you use the 40-hour route, 20 hours must be completed before the caregiver serves any client and the other 20 within six months. No caregiver may perform a task until training is done and competency is documented.

Companions and sitters have a lighter standard: they must be able to read, write, follow instructions, and complete reports, and must show competency in the areas the rule lists — understanding the needs of the elderly, handicapped, or convalescing; meal preparation and serving; transportation and escort; housekeeping and sanitation; home safety; handling medical emergencies in the home; and infection control. Everyone is oriented before serving clients and gets ongoing in-service training, and every direct-care worker is evaluated in writing at least once a year.

Part 4 — Setting Up to Operate

1. **Form your entity.** Register your LLC or corporation with the Georgia Secretary of State, Corporations Division (ecorp.sos.ga.gov; 1-844-753-7825), and get an EIN from the IRS.
2. **Register your trade name at the county.** Georgia files 'doing business as' names with the Clerk of the Superior Court in your county — not the state.
3. **Get insurance.** You need general and professional liability insurance to license. Carry workers' compensation once you have three or more employees — in Georgia, owners, officers, and LLC members count toward the three (this is a lower threshold than some states).
4. **Line up your registered nurse.** Employ or contract a Georgia-licensed RN to complete initial service plans for medically frail clients, direct your PCA training, and be reachable on call for those clients' needs.
5. **Set your rates and paperwork.** Build your service agreement, schedule of charges, client-rights statement, and forms (this package includes them).
6. **Know your wage-and-hour rules.** As an agency employer you pay at least the federal minimum wage of \$7.25/hour with overtime after 40 hours in a week — you cannot claim the 'companionship' exemption. Georgia has no daily overtime and no mandated breaks. Pay for compensable travel between clients. (Verify current figures.)

Part 5 — Finding Clients & Referrals

Your referral engine has two tracks: private-pay and Medicaid. Private-pay clients and families find you through your website, your reputation, and relationships with people who touch seniors at a moment of need — hospital discharge planners and social workers, skilled-nursing and rehab facilities, assisted living communities, hospices, elder-law attorneys, geriatric care managers, and senior-placement specialists. Build those relationships before you are licensed so referrals flow the day you open.

For Medicaid, the most important relationship in your county is not with the state — it is with your Area Agency on Aging (AAA). AAA case managers, together with the Georgia Division of Aging Services, refer clients into the CCSP and SOURCE waivers. Your companion Referral System edition gives you verified, named referral targets for Atlanta, Augusta, Savannah, and Columbus, organized by category, so you are not starting from a blank page.

Referral targets are in your Referral System edition

The Georgia edition of the Referral System lists real hospitals and discharge planners, skilled-nursing and rehab centers, hospices, assisted living communities, funeral homes, elder-law attorneys, and geriatric care managers — by metro, with names, addresses, and phone numbers — so you can start outreach immediately. Verify each contact before you rely on it.

Part 6 — Getting Paid

Private pay

The simplest revenue is private pay: the client or family pays you directly under your service agreement. There is no Medicaid enrollment and no Electronic Visit Verification for private-pay work, so most agencies launch on private pay while they pursue Medicaid.

Georgia Medicaid: the waivers

Georgia funds in-home personal care mainly through home and community-based waivers, not a broad state-plan benefit. The two that matter most are the Community Care Services Program (CCSP) and SOURCE, both under the Elderly and Disabled Waiver Program. SOURCE is CCSP's sister waiver — same population, with enhanced care coordination through the client's primary care physician. Personal Support Services (the hourly personal care benefit) and Structured Family Caregiving are delivered through these waivers.

Enrolling and billing

You enroll as a Medicaid provider through GAMMIS (mmis.georgia.gov), using your Georgia Medicaid ID and NPI, and then credential separately for the waiver — GAMMIS enrollment alone does not authorize waiver services. Two Georgia-specific gates trip up newcomers: the CCSP provider application is accepted only in March and September, and your DCH license must be at least nine months old before the required Provider Consult with your AAA. Plan your timeline around those windows.

EVV is mandatory for Medicaid personal care

Every Medicaid personal-care visit must be captured through Electronic Visit Verification. Georgia's state EVV vendor is Netsmart Mobile Caregiver+ (formerly Tellus) — not HHAEExchange or Sandata. It is free, and under Georgia's Open Choice model you may use a compliant system that integrates with the state aggregator. Your caregiver clocks in and out at the client's location; if the EVV record is missing or does not match the authorization, GAMMIS rejects the claim. The EVV support line is 833-483-5587.

Funding sources at a glance

Funding source	What it covers	How you get paid / verify
Private pay	Personal care, companion/sitter, home management under your service agreement	Direct billing to client/family; no EVV; verify your own rates
CCSP (EDWP waiver)	Personal Support Services and related in-home supports for frail elderly/disabled	Credential for CCSP (March/Sept), enroll in GAMMIS, bill with EVV; license must be 9 months old first
SOURCE (EDWP waiver)	Same personal care scope as CCSP with enhanced PCP care coordination	Credential via GAMMIS/care coordination; EVV through Netsmart
Structured Family Caregiving	Daily-rate support for family caregivers (inside CCSP/SOURCE)	Through an approved agency; reported around \$80/day — verify current
CMOs (Georgia Families)	Managed-care members: Amerigroup/Anthem, CareSource, Peach State	Amerigroup 1-800-249-0442 · CareSource 1-855-202-1058 · Peach State 1-800-704-1484
VA & LTC insurance	Veterans' home care benefits; long-term-care insurance policies	Verify eligibility and rates with the VA and the insurer

Dollar figures above are reported starting points, not quotes — rates reset periodically and vary by program and region. Verify current amounts with the payer before you rely on them.

Part 7 — Your First 90 Days Operating

1. Days 1–15: Confirm your provisional license is posted, your policies are in force, and your GCHEXS screening and training files are complete for every hire.
2. Days 1–30: Onboard your first caregivers — finish orientation and the first 20 hours of PCA training before anyone touches a client; document competency for each assigned task.
3. Days 15–45: Take your first clients on private pay; complete each initial assessment and service plan (RN completes plans for medically frail clients) before care begins.
4. Days 30–60: Stand up your documentation rhythm — visit notes every shift, supervisory-visit due dates on the calendar (92 days personal care / 122 days companion), and change-in-condition reporting.
5. Days 45–75: Build referral relationships — AAA case managers, hospital discharge planners, and facilities in your metro from your Referral System edition.
6. Days 60–90: Prepare for your on-site survey: self-audit client and personnel records, confirm every caregiver is screened, trained, and evaluated, and fix gaps now, not at the survey.
7. Ongoing: If pursuing Medicaid, track the March/September CCSP window and your 9-month license milestone; enroll in GAMMIS and set up Netsmart EVV.

Part 8 — Common Mistakes to Avoid

- **Serving clients before training is complete.** A PCA cannot perform a task before the required training and a documented competency determination — surveyors check this first.
- **Skipping the RN for frail clients.** For medically frail or compromised clients receiving personal care, an RN must complete the initial service plan. No RN, no compliant plan.
- **Missing supervisory home visits.** Personal-care clients need a supervisory visit at least every 92 days; companion/sitter clients at least every 122 days. Track due dates from day one.
- **Drifting into skilled nursing.** Administering medication or doing wound care crosses into home health. Reminders only — refer skilled needs out.
- **Missing the Medicaid windows.** CCSP applications are accepted only in March and September, and your license must be 9 months old first. Plan backward from those dates.
- **Billing without clean EVV.** A Medicaid personal-care visit without a matching Netsmart record is unbillable, no matter how well you documented it.
- **Letting the license lapse.** Miss the annual renewal and you pay the higher involuntary rate — and operating without a current license is prohibited.
- **Mishandling client funds.** No gifts, tips, borrowing, or acting as payee. Document every errand with receipts.

Part 9 — Staying Compliant Year-Round

- Renew your license on time each year; keep proof of payment and watch for the two-month renewal notice.
- Keep GCHEXS screening current and re-screen as required; never let a new hire start client contact on a pending check.
- Make supervisory home visits on the 92-day (personal care) and 122-day (companion/sitter) schedule and document each one in the client file.
- Evaluate every direct-care employee in writing at least annually through observation or demonstration.
- Keep your RN engaged for frail-client planning, PCA training direction, and on-call consultation.
- Run your quality assurance program: review incidents, grievances, documentation, and supervisory timeliness, and act on what you find.
- Keep Medicaid enrollment, waiver credentialing, and EVV setup current; reconcile EVV to authorizations before billing.
- Maintain workers' comp at three or more employees and keep your labor practices FLSA-compliant.

Worked Example 1 — Metro Atlanta

Maria wants to open a non-medical home care agency in Fulton County serving metro Atlanta. Here is her path.

- **Entity & setup:** She forms an LLC with the Georgia Secretary of State (ecorp.sos.ga.gov, 1-844-753-7825), gets an EIN, registers her trade name with the Fulton County Clerk of Superior Court, and binds general and professional liability plus workers' comp (she'll have three-plus staff).
- **RN & policies:** She contracts a Georgia-licensed RN for care planning and to direct her 40-hour PCA training, and adopts her Chapter 111-8-65 policy manual.
- **Screening:** She registers in GCHEXS (VoluntaryBackgroundCheck@dch.ga.gov) and fingerprints herself, her administrator, and her caregivers.
- **License:** She applies through GAHLES (gahles.dch.ga.gov), gets her provisional license, and starts serving private-pay clients while preparing for her on-site survey.
- **Referrals:** She connects with the Atlanta Regional Commission Area Agency on Aging / Empowerline (229 Peachtree St NE, Suite 100, Atlanta 30303; 404-463-3333), which serves Fulton and the surrounding metro counties, and with hospital discharge planners from her Referral System edition.
- **Medicaid:** She enrolls in GAMMIS, marks the next March or September CCSP window, notes her license must reach 9 months old before her Provider Consult, and sets up Netsmart EVV (support 833-483-5587).

Worked Example 2 — Savannah (Coastal Georgia)

James is opening in Chatham County, serving Savannah and the coast. His steps mirror Maria's, tuned to his region.

- **Entity & setup:** LLC with the Secretary of State, EIN, trade name filed with the Chatham County Clerk of Superior Court, and liability plus workers' comp coverage.
- **RN & training:** He engages a Georgia-licensed RN, then runs his 40-hour PCA program (20 hours before service, 20 within six months) under the RN's direction.
- **License & survey:** He files through GAHLES, receives his provisional license, becomes operational, and passes his on-site survey to earn the regular license.
- **Referrals:** He works with the Coastal Regional Commission Area Agency on Aging (1181 Coastal Drive SW, Darien 31305; 1-800-580-6860 / 912-437-0800), which serves Chatham and the surrounding coastal counties, plus Savannah-area hospital discharge planners and assisted living communities from his Referral System edition.
- **Getting paid:** He launches on private pay, then pursues CCSP/SOURCE through GAMMIS on the March/September cycle, billing with Netsmart EVV.

Frequently Asked Questions

Do I need a Certificate of Need?

No. A Certificate of Need is required for home health and hospice in Georgia, but not for a Private Home Care Provider. That is a big reason non-medical home care is the common entry point.

Can my caregivers do hands-on personal care?

Yes — that is the key difference from some other states. A Georgia PHCP license covers personal care (bathing, dressing, and other ADLs) delivered by trained Personal Care Assistants, in addition to companion/sitter and home-management services. What you cannot do is skilled nursing.

Do I have to be a nurse to own the agency?

No. You need a qualified administrator with health-related management experience, and a Georgia-licensed RN for care planning and training where the rules require it — but the owner does not have to be a nurse.

How fast can I start earning?

You can serve private-pay clients as soon as your provisional license is issued. Medicaid takes longer because of GAMMIS enrollment, waiver credentialing, the March/September CCSP window, and the 9-month license requirement.

What is the license fee?

The current fee is on the GAHLES application's fee schedule. Third-party guides commonly cite about \$350 for the initial application, but confirm the current amount on GAHLES before you budget — do not rely on a secondhand number.

Statewide Contacts

For...	Who to contact
Licensing the agency (PHCP)	GA Dept of Community Health, Healthcare Facility Regulation Division (HFRD) — (404) 656-4507; apply via GAHLES, gahles.dch.ga.gov ; hfrd.applicationswaivers@dch.ga.gov
Background checks	GCHXS (Georgia Criminal History Electronic System); facility access via VoluntaryBackgroundCheck@dch.ga.gov
Verify a license / find a facility	GaMap2Care (via dch.georgia.gov)
Form your business	Georgia Secretary of State, Corporations Division — ecorp.sos.ga.gov ; 1-844-753-7825
Trade name (DBA)	Clerk of Superior Court in your county
Medicaid enrollment & billing	GAMMIS — mmis.georgia.gov (Georgia Medicaid ID + NPI)
Waiver referrals (CCSP/SOURCE)	Your Area Agency on Aging / ADRC — 1-866-552-4464; aging.georgia.gov
Electronic Visit Verification	Netsmart Mobile Caregiver+ — support 833-483-5587; mobilecaregiverplus.com/ga-dch
Managed care (Georgia Families)	Amerigroup/Anthem 1-800-249-0442 · CareSource 1-855-202-1058 · Peach State 1-800-704-1484
Report abuse (Adult Protective Services)	1-866-552-4464 (after-hours 1-888-774-0152)
Workers' compensation	State Board of Workers' Compensation — (404) 656-3818 / 1-800-533-0682
Atlanta AAA	Atlanta Regional Commission / Empowerline — 404-463-3333
Savannah / coastal AAA	Coastal Regional Commission AAA — 1-800-580-6860 / 912-437-0800
Augusta AAA	CSRA Regional Commission AAA — (706) 210-2018
Columbus AAA	River Valley Regional Commission AAA — (706) 256-2910

Glossary

- **PHCP** — Private Home Care Provider, the Georgia license for non-medical in-home care.
- **HFRD** — Healthcare Facility Regulation Division of DCH; licenses and surveys your agency.
- **GAHLES** — Georgia Healthcare Licensing, Enforcement, and Surveys — the online portal for PHCP applications.
- **GCHXS** — Georgia Criminal History Electronic System — fingerprint background-check system.
- **PCA** — Personal Care Assistant — the caregiver who performs hands-on personal care after required training.
- **CCSP** — Community Care Services Program, Georgia's primary HCBS waiver for personal support.
- **SOURCE** — Sister waiver to CCSP with enhanced primary-care coordination.
- **EDWP** — Elderly and Disabled Waiver Program — the authority under which CCSP and SOURCE operate.
- **GAMMIS** — Georgia Medicaid Management Information System — provider enrollment and billing.
- **EVV** — Electronic Visit Verification; Georgia's vendor is Netsmart Mobile Caregiver+.
- **AAA** — Area Agency on Aging — your local hub for waiver referrals.
- **ADLs** — Activities of Daily Living — bathing, dressing, grooming, toileting, transferring, eating.

Core Rules & References

- Ga. Comp. R. & Regs. Chapter 111-8-65 — Rules and Regulations for Private Home Care Providers (definitions .03; licensing .05; inspections .08; administration & personnel .09; services & supervision .10; service plan .11).
- O.C.G.A. § 31-7-300 et seq. — the statute governing private home care providers (authority also at §§ 31-2-5, 31-2-7).
- O.C.G.A. § 30-5-1 et seq. — Disabled Adults and Elder Persons Protection Act (abuse reporting).
- O.C.G.A. § 34-9-2(a)(2) — workers' compensation required at three or more employees.
- Fair Labor Standards Act — minimum wage and overtime for agency employees (no companionship exemption for third-party employers).
- 21st Century Cures Act — Electronic Visit Verification mandate for Medicaid personal care.
- Georgia HCBS waivers — CCSP and SOURCE under the Elderly and Disabled Waiver Program; enrollment via GAMMIS.

Part 10 — Where to Go From Here

You now have the full path: what a Georgia Private Home Care Provider is, how to license through DCH/HFRD on GAHLES, how to screen and train under Chapter 111-8-65, how to set up your business, where clients come from, and how to get paid through private pay and the CCSP and SOURCE waivers. The next step is execution: form your entity, line up your RN, finish your policies and forms, run your GCHEXS screening, and file your GAHLES application.

Whatever pace you set, the fundamentals are constant: stay strictly within your non-medical scope, screen and train every caregiver, supervise on schedule, document everything, treat clients and staff with respect, and keep your compliance airtight. Do those things well and growth follows.

This guide is a general educational resource for prospective Georgia private home care providers. It is not legal, tax, or billing advice, and requirements, fees, and contacts change. Verify every step, figure, and phone number with DCH/HFRD, the Georgia Secretary of State, and the relevant payer before you rely on it. Provisions cited reflect Georgia law and DCH guidance at the time of writing.